

TO:	Local District Commissioners, Medicaid Directors
FROM:	Gabrielle Armenia, Director Division of Eligibility and Marketplace Integration
SUBJECT:	New Federal Eligibility Requirements for Non-Citizen Coverage in Medicaid and Child Health Plus (CHPlus)
ATTACHMENT:	Documentation Guide for Citizen and Non-Citizen Eligibility for Health Insurance Coverage in New York State (OHIP-0046)
EFFECTIVE DATE:	October 1, 2026
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Purpose

The purpose of this General Information System (GIS) message is to advise Local Departments of Social Services (LDSS) and New York City Human Resources Administration (HRA) of changes to Medicaid eligibility procedures resulting from new federal requirements involving immigration status.

Background

Section 71109 of Public Law 119-21, commonly referred to as "H.R.1," establishes significant new limits on the categories of non-citizens for whom federal financial participation (FFP) is available under Medicaid. These changes affect Medicaid coverage for certain lawfully present non-citizens who have historically been eligible for federally funded Medicaid coverage. As described below, these federal changes do not mean that individuals in these categories will no longer be eligible for health coverage in New York, but rather that federal funding is no longer available.

Before October 1, 2026, New York may receive FFP for Medicaid provided to "qualified" non-citizens, as defined under the Personal Responsibility and Work Opportunity Reconciliation Act of 1996 (PRWORA), who meet all other applicable eligibility requirements, as described in OHIP-0046, "Documentation Guide for Citizen and Non-Citizen Eligibility for Health Insurance Coverage in New York State."

Effective October 1, 2026, in addition to U.S. citizens and nationals, federal Medicaid funding is generally limited to individuals who are:

- Lawful permanent residents who have completed or are not subject to the 5-year bar;
- Cuban-Haitian entrants; or
- Compact of Free Association (COFA) migrants.

The new federal limitations do not affect New York's authority under section 214 of the Children's Health Insurance Program Reauthorization Act (CHIPRA) to provide federally funded Medicaid and Child Health Plus coverage to eligible lawfully residing children and pregnant individuals.

Affected non-citizens who are enrolled in Medicare may continue to receive Medicare coverage until federal Medicare eligibility changes take effect on January 4, 2027, but will no longer qualify

for the Medicare Savings Program (MSP) beginning October 1, 2026. Only citizens and the non-citizens in the groups listed above may qualify for the MSP after September 30, 2026.

The new federal Medicaid limitations also apply to Supplemental Security Income (SSI) recipients. Beginning October 1, 2026, SSI recipients must meet one of the above immigration categories to receive federally funded Medicaid. SSI recipients who do not meet these requirements will continue to receive Medicaid with State-only funding.

Procedure Change

Overview

The LDSS must distinguish between individuals whose Medicaid coverage will remain eligible for FFP and those whose coverage will be funded solely by New York State. To accommodate this distinction, systems changes are being made in the Welfare Management System (WMS) in the October migration, scheduled for October 19, 2026. These changes will allow district staff to more precisely distinguish immigration categories to align with federal law and to establish a new state/federal charge code. Additionally, the Department of Health (Department) will systematically be applying the new state/federal charge code to individuals whose eligibility was changed on or after October 1, 2026, but before the October 19, 2026, migration.

Systems changes include:

- A new WMS Alien Citizenship Indicator (ACI) code will be established to distinguish COFA migrants from refugees and Amerasians:
 - o Refugees and Amerasians will continue to be assigned ACI code **R**.
 - o COFA migrants will be assigned ACI code **Q**.
- A new state/federal charge code will be established for individuals who no longer qualify for FFP. State/federal charge code 78 (HR 1 FNP Adults (21+) Qualified Non-Citizens/HR1FNPAD) will be used for Medicaid recipients who are not identified as Lawful Permanent Residents (LPRs), Cuban-Haitian entrants, or COFA migrants.
 - o State/federal charge code 78 will be systematically applied to individuals with ACI A, B, D, G, J, M, R, or V when the charge code is blank or 88. The state/federal charge code will not be changed systematically to state/federal 78 if the worker enters a different valid charge code.

Refugees, Amerasians and COFA migrants all previously received ACI code R. Therefore, at renewal, LDSS staff must review information currently available in the case record for individuals with ACI code R to determine if the individual's ACI code needs to be updated to Q for COFA migrants. As described above, charge codes will be applied systematically, based on ACI to support proper federal claiming.

LDSS staff must verify if MSP applicants/recipients (A/Rs) meet the new immigration requirements for FFP when determining MSP eligibility from October 1, 2026, forward. If A/Rs are not citizens and do not meet the new immigration requirements for FFP, they are ineligible for MSP. As a reminder, individuals who do not qualify for FFP cannot receive MSP and State-only funding is not available for MSP.

Verification

The LDSS will continue to verify immigration status using documents provided by the individual

and are encouraged to use Attachment I “Documentation Guide for Citizen and Non-Citizen Eligibility for Health Insurance Coverage in New York State” (OHIP-0046) to determine what coverage and coding is appropriate, especially for non-citizens impacted by the changes discussed in this directive.

When documentation is needed to determine the individual's immigration status, a request for information must be made and the individual provided adequate time to respond to the request. If no response to this request is received, the individual's application for Medicaid must be denied or their Medicaid coverage discontinued.

Additionally, if documentation is received by the LDSS, and there is need to verify the legitimacy of the document, the Systematic Alien Verification for Entitlements (SAVE) system has been updated to provide information on whether an individual's immigration status qualifies for FFP under the new federal requirements.

Required Redeterminations for Existing Beneficiaries

The Department will systematically redetermine the eligibility of all potentially affected beneficiaries with coverage in WMS prior to October 1, 2026, to distinguish between individuals whose Medicaid coverage remains eligible for FFP and those whose coverage will be funded solely by New York State. On the fourth weekend of October, the Department will perform a conversion to systematically apply state/federal charge codes 65, 66, and 78 to consumers whose eligibility was determined prior to the October migration.

Non-citizens age 21 through 64 who no longer qualify for federally funded Medicaid will be transitioned to the Essential Plan (EP), if eligible, based on their immigration status and other applicable eligibility criteria. For additional information regarding the transition process to NY State of Health-administered Essential Plan coverage, see 16 OHIP/ADM-01.

Individuals who do not meet EP eligibility criteria will remain in WMS and receive Medicaid with State-only funding, if otherwise eligible. This may include lawfully present noncitizens who are under age 65 who require Long-Term Services and Supports (LTSS), such as nursing home care, or who qualify through a spenddown or pooled trust. However, some may transition to NY State of Health and remain in Medicaid with state-only funding because they have third-party health insurance coverage and cannot enroll in EP.

Required Redeterminations for Existing Medicare Savings Program Beneficiaries

Department staff are identifying existing MSP consumers on WMS who do not meet the new immigration requirements and will end their MSP effective September 30, 2026.

Consumers with both Medicaid and MSP will transition to State-only funded Medicaid beginning October 1, 2026, but will no longer be enrolled in MSP.

Consumers who have MSP-Only coverage will have their cases and Medicare Buy-In spans closed with an end date of September 30, 2026. Department staff will close the Medicare Buy-In spans for all impacted consumers and will also close the MSP-Only cases for Rest of State (ROS) consumers. However, HRA staff will need to close the MSP-Only cases for their consumers. The HRA MSP-only cases are expected to be a very small number of consumers (less than 50) and additional information will be provided to HRA to identify the affected consumers.

Department staff will also produce and mail notices to all impacted consumers who are losing MSP. Work will need to be undertaken in the near future to remove the MSP indicator from WMS for affected consumers. Additional guidance will be forthcoming regarding this work.

Please direct any questions to your local district liaison.